Patient Record — ID 1673

Age	30	**Sex**	F
Race/Ethnicity	Non-Hispanic White	**Insurance**	Blue Cross Blue Shield PPO
Total Encounters	8	**Generation Seed**	3426

Primary Diagnoses

• Second-degree perineal tear (healed)
• Diastasis recti abdominis
• Exclusive breastfeeding
• Lactational breast abscess
• Primary adrenal insufficiency
• Mastitis
• Normal spontaneous vaginal delivery
• Nipple fissure due to improper breastfeeding technique

Comorbidities

• Multiparity
• Exclusive breastfeeding
• Pregnancy (20 weeks gestation)
• Normal spontaneous vaginal delivery
• Lactation status

Patient Profile

Sex	F
Age at First Encounter	30
Race/Ethnicity	Non-Hispanic White
Insurance	Blue Cross Blue Shield PPO
Occupation	Elementary school teacher
Smoking Status	Never smoker
Alcohol Use	None (breastfeeding)

Allergies:

• NKDA

Chronic Conditions:

• Primary adrenal insufficiency
• Diastasis recti abdominis

Home Medications:

• Hydrocortisone - 20 mg PO qAM, 10 mg PO qPM
• Fludrocortisone - 0.1 mg PO daily

Surgical History:

• Repair of second-degree perineal tear (first delivery)
• Spontaneous vaginal delivery x2

Family History:

• Autoimmune hypothyroidism (mother)
• Rheumatoid arthritis (maternal aunt)
• Type 1 diabetes mellitus (paternal cousin)

Encounters (8)

Encounter 1 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	6811	**Type**	inpatient
Attending	Dr. Sarah Chen	**Order**	0
Department	Obstetrics & Gynecology	**Gen. Method**	template

Chief Complaint:

Regular contractions occurring every 5 minutes for the past two hours with bloody show

Full Note:

```
ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Sarah Chen, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Regular contractions occurring every 5 minutes for the past two hours with bloody show

HISTORY OF PRESENT ILLNESS:
She reports the contractions as moderately painful and notes the passage of a small amount of
mucoid, blood-tinged vaginal discharge. Her pregnancy has been uncomplicated, and prenatal care
has been adequate with no significant findings. The patient inquires about the significance of the
bloody show and the next steps in her management.

PAST MEDICAL HISTORY:
Active Problem List:
- Primary adrenal insufficiency
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)
```

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

VITAL SIGNS:
Blood pressure 120/78 mmHg, heart rate 88 bpm, respiratory rate 16 breaths/min, and temperature 98.6°F (37°C)

PHYSICAL EXAMINATION:
Fetal heart tracing is reassuring with a baseline of 140 bpm, moderate variability, no decelerations, and occasional accelerations. A sterile speculum examination confirms the presence of bloody show without active bleeding. Cervical examination reveals 4 cm dilation and 80% effacement.

Encounter 2 — 2020-01-15 — Obstetrics & Gynecology

Encounter ID	6805	**Type**	outpatient
Attending	Dr. Michael Rodriguez	**Order**	1
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Perineal pain and difficulty with urination 5 days after first vaginal delivery

Full Note:

```
ENCOUNTER NOTE
Date: 2020-01-15
Provider: Dr. Michael Rodriguez, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Perineal pain and difficulty with urination 5 days after first vaginal delivery

HISTORY OF PRESENT ILLNESS:
The patient with primary adrenal insufficiency and diastasis recti abdominis returns for follow-up
5 days after vaginal delivery of her first child. The delivery was complicated by a second-degree
perineal laceration, which was repaired immediately postpartum.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Primary adrenal insufficiency
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
She is breastfeeding and wishes to avoid medications that could affect her infant.

REVIEW OF SYSTEMS:
She denies fever, chills, or purulent discharge from the perineal area.

PHYSICAL EXAMINATION:
On examination, the perineum is edematous with mild erythema but without fluctuance or wound
dehiscence. The laceration repair appears intact.
```

Encounter 3 — 2020-02-15 — Obstetrics & Gynecology

Encounter ID	6812	**Type**	outpatient
Attending	Dr. Emily Johnson	**Order**	2
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Persistent bilateral nipple pain and abrasions while breastfeeding 1-month-old infant

Full Note:

```
ENCOUNTER NOTE
Date: 2020-02-15
Provider: Dr. Emily Johnson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Persistent bilateral nipple pain and abrasions while breastfeeding 1-month-old infant

HISTORY OF PRESENT ILLNESS:
The patient is a 30-year-old female with primary adrenal insufficiency and diastasis recti
abdominis who returns approximately one month following normal spontaneous vaginal delivery. She
presents with persistent breastfeeding difficulties. She describes the pain as sharp and burning,
which worsens during feeding. She also reports bilateral nipple abrasions and occasional bloody
nipple discharge. The patient has been exclusively breastfeeding and has not introduced bottle
feeding. The patient is concerned about the possibility of infection or underlying breast
pathology.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Second-degree perineal tear (healed) (diagnosed 2020-01-15)
- Primary adrenal insufficiency
- Diastasis recti abdominis

Her pregnancy was uncomplicated, and she had a normal spontaneous vaginal delivery.

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

REVIEW OF SYSTEMS:
She denies any fever, chills, or flu-like symptoms.
```

PHYSICAL EXAMINATION:
On physical examination, the nipples are erythematous with small cracks, but there is no evidence of mastitis or breast engorgement. No masses are palpable, and the infant appears well-nourished and active.

Encounter 4 — 2020-03-15 — Endocrinology

Encounter ID	6809	**Type**	outpatient
Attending	Dr. Aisha Patel	**Order**	3
Department	Endocrinology	**Gen. Method**	hybrid

Chief Complaint:

6 weeks postpartum with progressive fatigue, dizziness, 5kg weight loss, and skin hyperpigmentation

Full Note:

```
ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. Aisha Patel, MD
Department: Endocrinology
Visit Type: outpatient

CHIEF COMPLAINT:
6 weeks postpartum with progressive fatigue, dizziness, 5kg weight loss, and skin
hyperpigmentation

HISTORY OF PRESENT ILLNESS:
The patient presents 6 weeks postpartum after vaginal delivery. She reports that her skin has
become darker, especially in creases, scars, and her gums. She has been breastfeeding.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Second-degree perineal tear (healed) (diagnosed 2020-01-15)
- Nipple fissure due to improper breastfeeding technique (diagnosed 2020-02-15)
- Primary adrenal insufficiency
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

PHYSICAL EXAMINATION:
On physical examination, her blood pressure is 100/60 mmHg, and she has orthostatic hypotension.

LABORATORY DATA:
Laboratory tests reveal the following:

- Sodium: 130 mEq/L (Normal: 135-145 mEq/L)
- Potassium: 5.5 mEq/L (Normal: 3.5-5.1 mEq/L)
```

- Cortisol (8 AM): 3 µg/dL (Normal: 7-25 µg/dL)
- ACTH: 200 pg/mL (Normal: 10-60 pg/mL)

Encounter 5 — 2020-03-15 — Obstetrics & Gynecology

Encounter ID	6810	**Type**	outpatient
Attending	Dr. David Kim	**Order**	4
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Fever, malaise, and painful erythematous right breast for 4 days while breastfeeding

Full Note:

```
ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. David Kim, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Fever, malaise, and painful erythematous right breast for 4 days while breastfeeding

HISTORY OF PRESENT ILLNESS:
This 30-year-old female, 6 weeks postpartum with primary adrenal insufficiency, presents with
acute breast symptoms. She reports that the symptoms began with flu-like symptoms and localized
breast pain during breastfeeding. She was prescribed oral dicloxacillin 2 days ago but has not
experienced any improvement.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Second-degree perineal tear (healed) (diagnosed 2020-01-15)
- Nipple fissure due to improper breastfeeding technique (diagnosed 2020-02-15)
- Primary adrenal insufficiency (diagnosed 2020-03-15)
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
She has no known drug allergies

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

VITAL SIGNS:
On examination, her temperature is 38.7°C (101.7°F), heart rate is 88/min, and blood pressure is
120/78 mmHg.

PHYSICAL EXAMINATION:
The right breast has a 5 cm area of induration, warmth, and erythema in the upper outer quadrant
without fluctuance. There is no nipple discharge or retraction. Axillary lymph nodes are
```

non-palpable.

LABORATORY DATA:
Laboratory tests reveal:
- White blood cell count: 12,000/uL (normal: 4,500-11,000/uL)
- C-reactive protein: 18 mg/L (normal: <5 mg/L)
- Blood cultures: No growth to date

ASSESSMENT AND PLAN:
Given the lack of improvement with antibiotic therapy,

Encounter 6 — 2020-03-15 — Emergency Medicine

Encounter ID	6808	**Type**	ed
Attending	Dr. James O'Brien	**Order**	5
Department	Emergency Medicine	**Gen. Method**	hybrid

Chief Complaint:

3-day history of increasing left breast pain, swelling, fever, and chills in breastfeeding mother

Full Note:

```
ENCOUNTER NOTE
Date: 2020-03-15
Provider: Dr. James O'Brien, MD
Department: Emergency Medicine
Visit Type: ed

CHIEF COMPLAINT:
3-day history of increasing left breast pain, swelling, fever, and chills in breastfeeding mother

HISTORY OF PRESENT ILLNESS:
The patient is a 30-year-old female with primary adrenal insufficiency evaluated for right breast
pain earlier today, and presents to the emergency department with a 3-day history of increasing
pain and swelling in her left breast. She reports fever and chills since yesterday. She is
breastfeeding her 2-month-old infant.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Second-degree perineal tear (healed) (diagnosed 2020-01-15)
- Nipple fissure due to improper breastfeeding technique (diagnosed 2020-02-15)
- Primary adrenal insufficiency (diagnosed 2020-03-15)
- Mastitis (diagnosed 2020-03-15)
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

VITAL SIGNS:
Temperature is 38.7°C (101.7°F), heart rate is 102 beats per minute, and blood pressure is 120/78
mmHg

PHYSICAL EXAMINATION:
The left breast is erythematous and edematous with a 5 cm fluctuant mass noted in the upper outer
```

quadrant. There is associated tenderness on palpation and axillary lymphadenopathy on the same side. The right breast appears normal.

LABORATORY DATA:
White blood cell count: 14,000/mm^3 (normal 4,500-11,000/mm^3)
- Hemoglobin: 13.2 g/dL (normal 12-16 g/dL)
- C-reactive protein: 24 mg/L (normal <5 mg/L)

Encounter 7 — 2023-01-15 — Obstetrics & Gynecology

Encounter ID	6807	**Type**	inpatient
Attending	Dr. Lisa Wong	**Order**	6
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Routine postpartum evaluation 24 hours after uncomplicated vaginal delivery of second child

Full Note:

```
ENCOUNTER NOTE
Date: 2023-01-15
Provider: Dr. Lisa Wong, MD
Department: Obstetrics & Gynecology
Visit Type: inpatient

CHIEF COMPLAINT:
Routine postpartum evaluation 24 hours after uncomplicated vaginal delivery of second child

HISTORY OF PRESENT ILLNESS:
The patient with primary adrenal insufficiency returns for postpartum evaluation following her
second vaginal delivery, three years after her first pregnancy was complicated by lactational
breast abscess and adrenal insufficiency diagnosis. She presents 24 hours after an uncomplicated
vaginal delivery of a healthy full-term infant, reporting mild cramping and a small amount of
vaginal bleeding described as less than her normal menstrual period. She also mentions
experiencing intermittent chills and shivering, but denies any chest pain, shortness of breath, or
headache.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Second-degree perineal tear (healed) (diagnosed 2020-01-15)
- Nipple fissure due to improper breastfeeding technique (diagnosed 2020-02-15)
- Primary adrenal insufficiency (diagnosed 2020-03-15)
- Mastitis (diagnosed 2020-03-15)
- Lactational breast abscess (diagnosed 2020-03-15)
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

VITAL SIGNS:
Her vital signs are as follows: temperature 37.8°C (100°F), blood pressure 120/75 mmHg, heart rate
```

80 beats per minute, and respiratory rate 16 breaths per minute

PHYSICAL EXAMINATION:
Physical examination reveals a firm, non-tender uterus just below the level of the umbilicus and mild lochia rubra without clots. Her breasts are full and tender on palpation. The remainder of the examination is unremarkable.

Encounter 8 — 2023-03-15 — Obstetrics & Gynecology

Encounter ID	6806	**Type**	outpatient
Attending	Dr. Robert Thompson	**Order**	7
Department	Obstetrics & Gynecology	**Gen. Method**	hybrid

Chief Complaint:

Postpartum check-up 8 weeks after second delivery with abdominal midline bulge

Full Note:

```
ENCOUNTER NOTE
Date: 2023-03-15
Provider: Dr. Robert Thompson, MD
Department: Obstetrics & Gynecology
Visit Type: outpatient

CHIEF COMPLAINT:
Postpartum check-up 8 weeks after second delivery with abdominal midline bulge

HISTORY OF PRESENT ILLNESS:
The patient is a 33-year-old female with primary adrenal insufficiency who returns for routine
postpartum care. She gave birth to her second child 8 weeks ago via normal vaginal delivery. She
reports noticing a bulge in the midline of her abdomen that becomes more prominent when she is
sitting up or when she strains. She denies any pain associated with the bulge. Her pregnancy and
delivery were uncomplicated.

PAST MEDICAL HISTORY:
Active Problem List:
- Normal spontaneous vaginal delivery (diagnosed 2020-01-15)
- Second-degree perineal tear (healed) (diagnosed 2020-01-15)
- Nipple fissure due to improper breastfeeding technique (diagnosed 2020-02-15)
- Primary adrenal insufficiency (diagnosed 2020-03-15)
- Mastitis (diagnosed 2020-03-15)
- Lactational breast abscess (diagnosed 2020-03-15)
- Exclusive breastfeeding (diagnosed 2023-01-15)
- Diastasis recti abdominis

PAST SURGICAL HISTORY:
- Repair of second-degree perineal tear (first delivery)
- Spontaneous vaginal delivery x2

MEDICATIONS:
Home Medications:
- Hydrocortisone 20 mg PO qAM, 10 mg PO qPM
- Fludrocortisone 0.1 mg PO daily

ALLERGIES:
NKDA

FAMILY HISTORY:
- Autoimmune hypothyroidism (mother)
- Rheumatoid arthritis (maternal aunt)
- Type 1 diabetes mellitus (paternal cousin)

SOCIAL HISTORY:
Smoking: Never smoker. Alcohol: None (breastfeeding). Occupation: Elementary school teacher.

VITAL SIGNS:
Her vital signs are within normal limits.
```

PHYSICAL EXAMINATION:
On examination, a midline protrusion is noted above the umbilicus when she raises her head while lying down. Palpation of the abdomen does not elicit tenderness, and the protrusion does not change with coughing. There is no evidence of a hernia on examination.